

CANDIDATE CAPABILITY MATCH REPORT

August 2026

Renata Kowalczyk-Byrne

Registered Nurse – Intensive Care Unit (ICU)

vs. Registered Nurse – Intensive
Care Unit (ICU)

Renata Kowalczyk-Byrne

Silver Spring, MD 20904

Leadership · Staff development ·
Quality improvement leadership ·
Patient satisfaction · Communication
· Preceptor · Electronic Health Record
Inpatient Superuser · Telemetry
monitoring

Bachelor of Science in Nursing ·
Chesapeake State University

OVERALL CAPABILITY MATCH

45.71%

Against 14 evaluated capabilities

4

STRONG

2

PARTIAL

8

GAPS

REJECT · HIGH

We do not recommend proceeding with Renata Kowalczyk-Byrne for this role

The candidate lacks critical, high-acuity ICU skills and the required continuous ICU experience for this independent-practice travel assignment, making them unsuitable for a role requiring productivity after two shifts.

Capability match — criteria breakdown

#	Capability	Domain	Verdict	Score	Evidence / gap
1	Maintain active, unencumbered RN license (Maryland or compact)	Technical Skills	STRONG	8/10	Registered Nurse (certification listed)
2	BLS certification (AHA)	Technical Skills	STRONG	8/10	Basic Life Support (BLS) (certification listed)
3	ACLS certification (AHA)	Technical Skills	STRONG	8/10	Advanced Cardiovascular Life Support (certification listed)
4	Adult ICU experience (2+ years recent, continuous)	Technical Skills	GAP	3/10	Cross-trained to the intensive care unit in January 2025; float to the ICU approximately 1 shift per pay period. (This indicates recent ICU exposure, not 2+ continuous).
5	Manage ventilated patients	Technical Skills	GAP	0/10	No explicit mention of managing ventilated patients. Progressive care and monitoring experience typically do not involve ventilator management.
6	Manage vasoactive-dependent patients	Technical Skills	GAP	0/10	No explicit mention of managing vasoactive-dependent patients. This is a competency typically found in progressive care.
7	Perform head-to-toe assessments	Technical Skills	PARTIAL	7/10	As a Registered Nurse with 9 years of experience in acute care, performing assessments is a fundamental and continuous part of the role.
8	Interpret	Technical Skills	GAP	2/10	Telemetry monitoring, Rhythm interpretation

	hemodynamic data	Skills			mentioned, which are basic components of hemodynamic assessment, but more advanced interpretation required for arterial lines, CVP, cardiac output.
9	Titrate vasoactive, sedative, and analgesic infusions	Technical Skills	GAP	0/10	No evidence of titrating these types of infusions which are critical ICU skills.
10	Recognize and escalate clinical deterioration	Technical Skills	PARTIAL	7/10	Managed staffing, admissions flow, and escalation to the rapid response team indicates experience in recognizing and escalating deterioration.
11	Initiate and monitor Continuous Renal Replacement Therapy (CRRT)	Technical Skills	GAP	0/10	No evidence of CRRT experience.
12	Execute sepsis bundle elements	Technical Skills	GAP	0/10	No explicit mention of sepsis bundle.
13	Prioritize competing demands in 1:2 patient assignment	Technical Skills	GAP	5/10	Carried a 4 to 6 patient telemetry unit when not in the charge role. As a result, coordinated assignments for 9 to 10 patients. This shows prioritization skills, but not specifically in a 1:2 high-acuity ICU setting.
14	Electronic Health Record (EHR) experience	Tooling And Workflow	STRONG	9/10	Electronic health record documentation, Electronic Health Record Inpatient Superuser. This indicates deep experience.

Key strengths to leverage

Areas to probe in screen

Key strengths to leverage

- Strong EHR expertise (Superuser)
- Excellent foundational certifications (RN, BLS, ACLS)
- Proven ability to recognize and escalate clinical deterioration
- Extensive charge nurse experience
- Strong communication and leadership skills

Gaps to probe in screen

- Limited recent, continuous adult ICU experience
- No evidence of managing ventilated patients
- No evidence of managing vasoactive-dependent patients
- Lack of CRRT experience
- Limited advanced hemodynamic interpretation skills

Recommended probe questions for Registered Nurse – Intensive Care Unit (ICU)

#	Question	Area
Q1	The job requires independent management of ventilated patients. Can you describe your most significant experience managing a patient on a mechanical ventilator, including your role in ventilator weaning trials and extubation readiness assessments?	Ventilator Management Gap
Q2	This role involves titrating vasoactive infusions like norepinephrine and vasopressin. Please walk me through a time you managed a patient on multiple vasoactive drips, detailing your assessment, titration decisions, and how you monitored for	Vasoactive Management Gap

	effectiveness and side effects.	
Q3	Our ICU frequently initiates and monitors Continuous Renal Replacement Therapy (CRRT). What is your experience with CRRT, including initiating the circuit, troubleshooting common issues, and managing fluid balance for these patients?	CRRT Experience Gap
Q4	The job requires executing sepsis bundle elements within specific time windows. Describe a situation where you identified a patient with suspected sepsis and the steps you took to initiate and complete the sepsis bundle, including any challenges you faced.	Sepsis Bundle Execution
Q5	As a charge nurse, you've managed staffing and patient flow. How do you approach prioritizing competing demands when you have a 1:2 patient assignment, especially when one patient is unstable or requires intensive interventions like CRRT or post-arrest care?	Prioritization & Workload
Q6	You have extensive experience with EHRs, including being a Superuser. Can you share an example of how your EHR expertise has directly contributed to improved patient care or workflow efficiency in a critical care setting?	EHR Expertise Validation

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